

Acute Interstitial Nephritis

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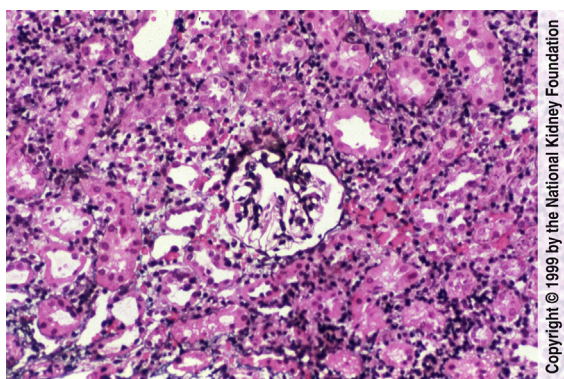


Fig 1. In acute interstitial nephritis, there is edema associated with an interstitial lymphoplasmocytic infiltrate. There are numerous causes for acute interstitial nephritis, including toxins, infections (especially viral), and most commonly in renal biopsy practice, drug-induced hypersensitivity reactions. The glomeruli are uninvolved, unless there is an associated minimal change disease-type injury, most often seen with drug-induced acute interstitial nephritis caused by nonsteroidal antiinflammatory drugs. (Jones' silver stain, original magnification $\times 100$).

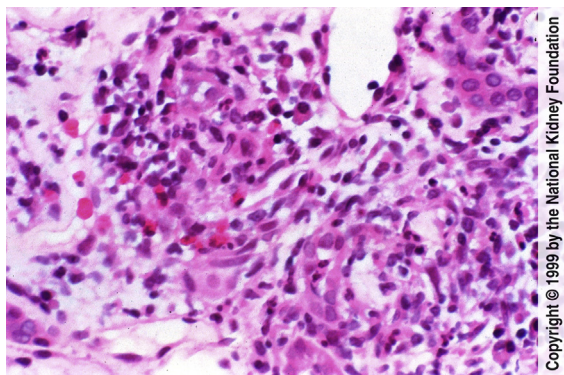


Fig 2. Eosinophils are frequently prominent in cases of acute interstitial nephritis because of drug-induced hypersensitivity reactions. Eosinophils are recognizable by the brightly eosinophilic granular cytoplasm on hematoxylin and eosin stains, and their bilobed nuclei. There is also associated interstitial edema. (Hematoxylin and eosin, original magnification $\times 400$).

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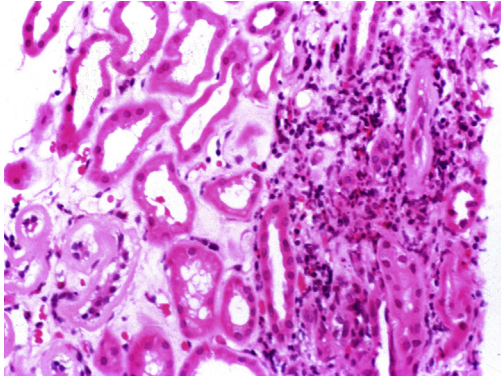
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Fig 3. There is edema in addition to preexisting mild tubulointerstitial fibrosis in this case of acute interstitial nephritis caused by drug-induced hypersensitivity. There is a prominent interstitial eosinophilic component, in addition to lymphocytes and plasma cells. (Hematoxylin and eosin, original magnification $\times 200$).